

ENSLAVEMENT, ABORTION, AND "RACE SUICIDE"

White Supremacist Reproductive Control in the Modern
Era

EMILIA RAVOILA

Milton Academy '26

Abortion is often defined as a modern-day issue. While it is true that the last fifty years have been significant in the debate over reproductive rights, abortion has been contentious in the United States since before the country's founding. In the antebellum United States, enslavers controlled enslaved women's reproduction in order to expand the institution of slavery. Midwives, many of them women of color, initially provided the majority of reproductive care on plantations. However, because midwives were more likely to participate in resistance against slavery, such as by providing abortions, white male doctors replaced the majority of midwives by the late nineteenth century. After the abolition of slavery in 1865, white male doctors saw how lucrative obstetrics and gynecology could be and sought to monopolize the industry, pushing midwives out of the field and using abortion bans as a strategy to do so. The eugenics movement of the late 1800s, fueled by white supremacy and fears of "race suicide"—the imagined end of the white race—motivated further reproductive control and restrictions. Additionally, the xenophobic response to increased immigration to the U.S. in the late 1800s and early 1900s exacerbated this control. White supremacist ideologies initiated reproductive restrictions in the U.S.; this motivation began with control over reproduction during slavery, continued with medicalization of reproduction, and persisted within the eugenics and anti-immigration movements in the nineteenth and twentieth centuries.

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In a pre-Civil War America with an economy built upon the labor of enslaved people, midwives provided essential reproductive care. The first enslaved people in the U.S. colonies were "forced on to Virginia's shores by a Dutchman in 1619." While European indentured labor had been the norm in the colonies up until that point, "by 1675 slavery

was well established...With plentiful land and slave labor available to grow a lucrative crop, southern planters prospered, and family-based tobacco plantations became the economic and social norm.” Bacon’s Rebellion in 1676 solidified slavery on a governmental level. Mary Elliott and Jazmine Hughes write that “in the wake of Bacon’s Rebellion, in which free and enslaved black people aligned themselves with poor white people...against the government, more stringent laws were enacted that defined status based on race and class. Black people in America were being enslaved for life, while the protections of whiteness were formalized.” With legislation like this, slavery became a form of racialized control of people and their labor, and, as a result, reproduction became a crucial issue. Reproduction served to protect the future of slavery; thus, enslavers sought to control it. Abortion didn’t become a nationally contentious issue in the United States until the 1800s. Annalies Winny writes that before then, “the U.S. attitude toward abortion was much the same as it had often been elsewhere throughout history: It was a quiet reality, legal until ‘quickening’ (when fetal motion could be felt by the mother). In the eyes of the law, the fetus wasn’t a ‘separate distinct entity until then,’ but rather an extension of the mother.” Reproductive care was largely unregulated—skilled midwives provided essential care. Midwifery was interracial, as half of reproductive healthcare providers were Black women, and others were Indigenous and white. “In colonial times, midwifery reflected women from all backgrounds, including enslaved Black women, who worked as enslaved midwives in the antebellum South...midwives learned through apprenticeship from experienced, older midwives. They traveled to homes to care for mothers and their babies. Midwives were critical to their communities and often the only primary care providers.” In the context of the institution of slavery, midwives were essential.

Extensive control over reproduction by enslavers and by governments allowed for the strengthening and perpetuation of the institution of American slavery. In 1662, legislators in Virginia passed a law determining that “all children borne in this country shalbe held bond or free only according to the condition of the mother.” This law made slavery legally hereditary and made it profitable for white enslavers to get the women they enslaved pregnant. Reproduction was therefore the foundation of the perpetuation of slavery. On January 1, 1808, the Act to Prohibit the Importation of Slaves took effect federally, making it illegal to import enslaved people into the United States. This furthered the impacts of the 1662 legislation—with no option to import slaves, enslavers further depended on reproductive control over enslaved women. As sociologist Rodney Coates writes, “with this shift, enslavers stepped up the forced breeding of enslaved women. White men raped

the Black women and girls they enslaved, and then enslaved the children born from those rapes. White men also forced the Black women and Black men they enslaved to have sex with one another to generate more babies, who would be born into slavery." Enslavers saw how vital reproduction was to the institution of slavery, and as a result they sought to control it. Founding father, slave owner, and President Thomas Jefferson highlighted this abuse by enslavers in an 1819 letter, stating, "a child raised every 2 years is of more profit than the crop of the best laboring man... it is not their labor, but their increase which is the first consideration with us." Slavery was built upon the reproduction of Black women, and enslavers sought to capitalize on slavery by controlling that reproduction.

Enslaved women found ways to resist this control of their bodies; many of these methods of resistance were supported and led by midwives. "Enslaved women understood that their childbearing and rearing lay at the foundation of slavery," writes historian Signe Fourmy. "Consequently, preventing pregnancy or terminating a pregnancy represented more than an exercise of bodily autonomy, which was an act of resistance in and of itself; it represented a refusal to perform the labor deemed necessary for enslaved women." "The social value of midwives' roles cannot be underestimated under the oppressions of slavery," write sociologists Keisha Goode and Barbara Katz Rothman. On the historical significance of midwives, the National Museum of African American History and Culture notes that "early African American midwives were important members of their community, even among enslaved individuals. Slave owners used these medical practitioners to ensure the health of their reproducing enslaved women and their newborn infants to expand their labor force." Midwives were integral in the plantation system because they provided essential care to enslaved women. Sometimes, this essential care included providing abortion and contraceptive services without the knowledge of enslavers, a practice which enslavers sought to disrupt. One such instance was in 1860 in Rutherford County, Tennessee, when enslavers and doctors hoping to remedy the enslaved women in the county's lack of reproduction met to discuss the issue. Politician John T. Morgan stated that efforts "to effect an abortion" included using "tansy, rue, roots and seeds of the cotton plant, pennyroyal, cedar gum, and camphor." Enslaved women, many led by midwives, found ways to resist the dehumanization and commodification of their bodies through enslavement by refusing to have children. The ensuing debate in Rutherford County "centered on whether instances of infertility resulted from enslaved women's 'want of attention and care,' their harsh working conditions, or, perhaps even more troubling to the planters and doctors, deliberate interference," explains Fourmy. An anonymous doctor told of a man who enslaved "four

to six slave women of the proper age to breed for twenty-five years” to whom “only two children had been born.” “Eventually,” writes Fourmy, “the enslaved women confessed and identified ‘the weed’ they used to obtain the desired effect. The same physician recounted that on another plantation an “old negro woman” admitted to having supplied ‘the remedy’ to fellow bondwomen.” Contraception and abortion represented a resistance to slavery for many enslaved women, and midwives helped to provide those services.

Because midwives often provided contraception and abortion services to enslaved women, enslavers began to lose trust in them and turn to white male doctors—this shift on plantations was in fact the violent origin of modern gynecology. Coates writes that enslavers made this change “to figure out why nearly half of enslaved infants were stillborn or died within their first year of life and why so many enslaved women were infertile.” They suspected that midwives had something to do with it. White male doctors soon attained a larger role in the plantation system. “Physicians collaborated with slave owners in efforts to increase fertility among slaves...Obstetric cases allowed physicians the opportunity to develop a burgeoning practice by experimenting and acquiring skills on the bodies of enslaved women,” write Goode and Rothman. This abusive involvement shaped the future of gynecology. White doctors not only “participated in the maintenance of the plantation regime and of labor increase by overseeing and interfering in the births of enslaved children,” explains historian Crystal Webster, but also “violently experimented on Black women without anesthesia or consent.” One of these white doctors, J. Marion Sims, known as the “father of modern gynecology,” conducted much of his research on slaves who were rarely given anesthesia. “Physician dominance of birth was beginning to take a strong hold,” write Goode and Rothman, “while in the beginning of the 19th century the majority...of white plantation births were attended by midwives...by 1860, an estimated 40-50 percent of plantation mistresses were delivered by physicians.” While midwives initially provided care to enslaved women, white male doctors soon replaced them to increase enslavers’ control—this was the origin of modern gynecology.

From the 1800s to the 1900s, the U.S. saw a larger shift: the medicalization of reproductive care, fueled by racism, misogyny, and economic greed; this medicalization began a larger trend of abortion bans and reproductive restrictions in the country and was deeply rooted in enslavement. Although white male doctors had started to replace midwives in the plantation system in the early nineteenth century, midwives still provided the vast majority of reproductive care in the United States through the early 1900s. “Skilled Black midwives represented both real competition for white men who sought to

enter the practice of child delivery, and a threat to how obstetricians viewed themselves. Male gynecologists claimed midwifery was a degrading means of obstetrical care. They viewed themselves as elite members of a trained profession with tools such as forceps and other technologies, and the modern convenience of hospitals, which excluded Black and Indigenous women from practice within their institutions," writes legal scholar Michele Goodwin. These physicians, who had learned much of their knowledge from violent and cruel experiments on enslaved women, felt threatened by midwives, especially Black midwives. The Planned Parenthood Action Fund has reported that "starting around the time of the Civil War, a coalition of male doctors...led a movement to push state governments to outlaw abortion across the board. The male-dominated medical profession wanted to take authority from the female-dominated profession of midwives, including the authority to provide abortion." White male doctors, many of whom were prejudiced against these midwives, saw how lucrative obstetrics could be. One institution, the American Medical Association, established in 1847, saw abortion as an easy target. "Their rhetoric was strategic," says Mary Fissell, a History of Medicine Professor at Johns Hopkins University. "You have to link those midwives to providing abortion as a way of kind of getting them out of business," Fissell says. "So organized medicine very much takes the anti-abortion position." White physician Horatio Storer wrote in 1866 that "induced abortions are not only a crime against life...against the mother...against nature and all natural instinct, and against public interests and morality, but that...they are so dangerous to the woman's health, her own physical and domestic best interests, that their induction, permittal, or solicitation by one cognizant of their true character, should almost be looked upon as proof of actual insanity." Storer, who was motivated by racist beliefs, made false statements such as this one in order to further restrict women's access to reproductive care. Fissell writes that "in 1857, the AMA took aim at unregulated abortion providers with a letter-writing campaign pushing state lawmakers to ban the practice. To make their case, they asserted that there was a medical consensus that life begins at conception, rather than at quickening...The campaign succeeded. At least 40 anti-abortion laws went on the books between 1860 and 1880." Such efforts by the medical community were the deciding factor in the initial banning of abortion in the United States. The Virginia State Board of Health wrote in a 1913 letter that if "the midwife is shown beyond question to be a source of danger and death, then the state cannot refrain from stepping in and protecting her citizens." Doctors were the force that brought about the first large-scale abortion bans and reproductive restrictions, effectively pushing out midwives from gynecology and medicalizing the field.

Doctors' efforts to ban abortion, and therefore replace midwives, were largely successful, but they continued to try to monopolize reproductive care by creating and perpetuating discriminatory misinformation and stereotypes against midwives. In the 1900s, these doctors began to carry out "successful racist and misogynistic smear campaigns, cleverly designed for political persuasion and to achieve legal reform," writes Goodwin. Physician Joseph Delee stated in a 1916 speech that "the midwife destroys obstetric ideals... The midwife is a relic of barbarism. In civilized countries the midwife is wrong, has always been wrong... The midwife has been a drag on the progress of the science and art of obstetrics. Her existence stunts the one and degrades the other." White male doctors continued to spread misinformation about midwives in order to monopolize care. In his 1926 journal article titled "The Development of Midwifery in Mississippi," Dr. Felix Underwood, a physician and the director of the Bureau of Child Hygiene for Mississippi, wrote, "what could be a more pitiable picture than that of a prospective mother housed in an unsanitary home and attended in this most critical period by an accoucheur, filthy and ignorant, and not far removed from the jungles of Africa, laden with its atmosphere of weird superstition and voodooism." Doctors were explicitly trying to undermine midwives, ban abortion, and perpetuate white supremacy, and they used racist rhetoric and stereotypes to further their agenda. Their strategy was successful: "gynecologists pushed women out of the field of reproductive health... Doing so not only undercut women's reproductive health, but also drove qualified Black women out of medical services," writes Goodwin. "Gynecologists explicitly revealed their motivations in undermining midwifery: They desired financial gains... and a monopoly. As DeLee wrote in a 1916 article published in the *American Journal of Obstetrics & Disease of Women & Children*, 'There is high art in obstetrics and... it must pay as well'... [Doctors] believed that men should be paid, but not women — particularly not Black women." White male doctors saw how lucrative obstetrics were, and they advocated against midwives, creating racist and misogynistic stereotypes and successfully pushing Black midwives out of obstetrics.

After white male doctors had successfully outlawed abortion in the US, the eugenics movement, which stemmed in slavery and white supremacy, furthered reproductive restrictions, especially on women of color. "The goal of eugenic science was knowledge of how various traits – emotional, physical, intellectual – were inherited, so that such information could be applied in order to advance the human race and preserve imagined racial superiority," writes University of Virginia Professor of Medicine P. Preston Reynolds. "While Jefferson is credited with the language, 'all men are created equal,'" Reynolds

Enslavement, Abortion, and "Race Suicide": Modern White Supremacist Reproductive Control writes, "he also argued 'any attempt to assimilate [blacks] with the American polity is a greater threat to the integrity of the republic than naturalizing immigrants.' Jefferson paved the way for eugenicists by providing a rationale that harmonized their theories... Just as Jefferson argued that 'self preservation' was the nation's highest moral imperative, and its 'first law of nature,'... eugenicists sought to deprive the procreative liberties of blacks, poor whites and the mentally defective to prevent them from destroying the lives of other Virginians (particularly, affluent whites) through genetic pollution." This racist ideology originated in the slavery era. Eugenics was a continuation of the white supremacist views that had attempted to justify enslavement for centuries. Storer wrote in 1866 that "abortion... strikes a blow at the very foundation of society itself," expressing his fears that white women weren't reproducing enough. These same eugenicist arguments were pervasive in the early 1900s, pushing governments to further regulate access to reproductive care. At the turn of the twentieth century, infertility became an obsession for the eugenics movement, and in 1903, President Theodore Roosevelt spoke openly about "race suicide," a perceived threat that white people faced in losing the birthing numbers game to immigrants and minorities. "From 1907 to 1932, 32 states passed explicit eugenics laws that allowed for the government to sterilize the 'insane,' the 'feeble-minded,' the 'dependent' and the 'diseased.'" Many of these laws were used as an excuse to sterilize racial minorities, taking away people's ability to have children. The Equal Justice Initiative finds that these forced sterilization laws were "used as a tool of racialized population control. From the 1920s to 1970s, thousands of poor, Southern Black women were sterilized without their knowledge or consent... compulsory programs sterilized an estimated 65,000 individuals in more than 30 states, and the number is likely much higher." Virginia's 1924 Racial Integrity Act, yet another example of eugenicist policy, banned interracial marriage, defining a white person as having 'no trace of the blood of another race.'" This legislation further demonstrates people's fear of the idea of "race suicide." The eugenics movement, based in long-established white supremacist ideologies, perpetuated efforts to limit women of color's reproduction by forcing sterilization and increase white women's reproduction by banning abortion.

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These efforts to control people's reproduction were not only "justified" by eugenics but also by xenophobic and anti-immigrant beliefs—again, attempts to curb access to reproductive care were motivated by white supremacist ideas. Historians Ramtin Arablouei and Rund Abdelfatah explain that when Dr. Storer had argued against abortion in the 1800s, "he had fears of... what was commonly referred to as race suicide, that the [white population] wasn't going to replenish itself fast enough to keep up with the swells of new

immigrants to the United States.” Goodwin writes that “the racist campaigns launched by doctors against Black midwives extended to anti-immigration legislative platforms targeted at Chinese and Japanese workers. The Page Act, which restricted Chinese women from entering the United States, is a part of this shameful legacy...DeLee and...Storer urged white women to ‘spread their loins’ across the nation, a dog whistle about the threat of too many Blacks and Asians in the U.S.” Xenophobia and fears of a decreasing white population contributed to reproductive restrictions. Alex Samuels and Monica Potts write that “laws limiting abortion, it was believed, would ultimately force middle- and upper-class white women...to bear more white children...this helped bolster flawed, racist arguments for a total ban of the procedure.” The anti-immigration movement, deeply rooted in white supremacy, pushed bans on abortion and reproductive restrictions in the early 1900s.

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White supremacy has informed almost every reproductive restriction in United States history. White supremacist control over enslaved people’s reproduction during slavery, racist attacks against midwifery, and later eugenics and anti-immigration arguments all enabled abortion bans and reproductive restrictions. The subjugation of bodies in the United States, specifically of women of color, has a centuries-long legacy that did not end with the abolition of slavery. Today, huge inequities in access to abortion and reproductive care hurt low income and marginalized groups. The contemporary debate over abortion bans echoes the debates over slavery in the eighteenth and nineteenth centuries. “Like the battle over slavery, the fight over reproductive freedom raises questions about federal and state authority — in other words, who gets to make the rules,” writes historian Kate Masur. The overturning of *Roe v. Wade* “has created for pregnant women an analogous situation to the fugitive slave laws...where women may be prosecuted by the state in which they reside for obtaining an abortion beyond its borders. It will create legal chaos, triggering inter-state conflict, as fights over extradition and a state’s legal rights beyond its own borders will certainly erupt once more,” writes University of London Professor Sarah Churchwell. The debate over abortion today is reminiscent of the debates over slavery from centuries ago, because the two issues are very much intertwined: abortion bans have historically been rooted in white supremacy. Both debates, similarly to other contentious issues in our country’s history, have been driven by the desire of white men to maintain power.

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